

Stelara/Wezlana (ustekinumab) Subcutaneous (subQ)
Coverage Policy

Policy Title: Clinical Review Criteria Stelara/Wezlana (ustekinumab) SubQ	Effective Date: 6/14/2022
Policy Number: PS932POL	Revision Date: 6/17/2025
Lines of Business: Commercial	

HNE Approved Indications for use

STELARA/Wezlana is a human interleukin-12 and -23 antagonist indicated for the treatment of:

- Adult patients with:
 - moderate to severe plaque psoriasis (Ps) who are candidates for phototherapy or systemic therapy
 - active psoriatic arthritis (PsA), alone or in combination with methotrexate.
 - moderately to severely active Crohn's disease (CD)
 - moderately to severely active ulcerative colitis
- Pediatric patients 6 years and older with:
 - moderate to severe plaque psoriasis (Ps) who are candidates for phototherapy or systemic therapy
 - active psoriatic arthritis

The following criteria must be met for coverage of Stelara (ustekinumab) SubQ:

Criteria for coverage:

Prescriber must provide documentation of ALL of the following:

Plaque Psoriasis

- Member is ≥ 6 years of age **AND**
- Prescribed by or in consultation with a dermatologist **AND**
- Diagnosis of generalized moderate to severe chronic plaque psoriasis involving $\geq 10\%$ of body surface area **AND**
- Member has had a minimum duration of a 4-week trial and failure, or intolerance to **ONE** of the following topical therapies or contraindication to **ALL** of them:
 - Corticosteroids
 - Vitamin D analogs
 - Tazarotene
 - Calcineurin inhibitors
 - Anthralin
 - Coal tar **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), secukinumab (Cosentyx), golimumab (Simponi), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), upadacitinib (Rinvoq), adalimumab (Humira), tocilizumab (Actemra), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), apremilast (Otezla)]

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Psoriatic Arthritis

- Member is ≥ 6 years of age **AND**
- Prescribed by or in consultation with a dermatologist or rheumatologist **AND**
- Diagnosis of active psoriatic arthritis **AND**
- Member must be intolerant or failed a trial of at least one (1) of the following: a DMARD (i.e., methotrexate, sulfasalazine, hydroxychloroquine, aurothioglucose, auranofin, gold sodium thiomalate, azathioprine, d-penicillamine, cyclosporine, leflunomide **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), secukinumab (Cosentyx), golimumab (Simponi), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), upadacitinib (Rinvoq), adalimumab (Humira), tocilizumab (Actemra), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), apremilast (Otezla)]

Crohn's Disease

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a gastroenterologist **AND**
- Diagnosis of Crohn's disease **AND**
- Documented moderate to severe active disease **AND**
- Member has tried and failed or was intolerant to at least **ONE** of the following conventional therapies or has a contraindication to **ALL** of the following:
 - 6-mercaptopurine **OR**
 - Azathioprine **OR**
 - an oral corticosteroid (e.g. prednisone) **OR**
 - methotrexate **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), secukinumab (Cosentyx), golimumab (Simponi), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), upadacitinib (Rinvoq), adalimumab (Humira), tocilizumab (Actemra), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), apremilast (Otezla)]

Ulcerative Colitis

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a gastroenterologist
- Diagnosis of moderate to severe ulcerative colitis **AND**
- Member has tried and failed or was intolerant to at least **ONE** of the following conventional therapies or has a contraindication to **ALL** of the following:
 - 6-mercaptopurine **OR**
 - An aminosalicylate (e.g. mesalamine, olsalazine, sulfasalazine) **OR**
 - Azathioprine **OR**
 - an oral corticosteroid (e.g. prednisone) **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia),

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secukinumab (Cosentyx), golimumab (Simponi), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), upadacitinib (Rinvoq), adalimumab (Humira), tocilizumab (Actemra), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), apremilast (Otezla)]

Criteria for continuation of therapy:

Prescriber must provide documentation of ALL of the following:

Plaque Psoriasis

- Member is ≥ 6 years of age **AND**
- Prescribed by or in consultation with a dermatologist **AND**
- Diagnosis of plaque psoriasis **AND**
- Member must have demonstrated a clinical response to therapy (i.e. reduction in body surface area involvement or improvement in PASI score) **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), secukinumab (Cosentyx), golimumab (Simponi), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), upadacitinib (Rinvoq), adalimumab (Humira), tocilizumab (Actemra), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), apremilast (Otezla)]

Psoriatic Arthritis

- Member is ≥ 6 years of age **AND**
- Prescribed by or in consultation with a dermatologist or rheumatologist **AND**
- Diagnosis of psoriatic arthritis **AND**
- Member must have demonstrated a clinical response to therapy (i.e. improvement in physical function, control of the progression of joint damage, and/or pain reduction) **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), secukinumab (Cosentyx), golimumab (Simponi), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), upadacitinib (Rinvoq), adalimumab (Humira), tocilizumab (Actemra), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), apremilast (Otezla)]

Crohn's Disease

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a gastroenterologist **AND**
- Diagnosis of moderate to severe Crohn's disease **AND**
- Member demonstrated a clinical response to therapy (i.e., decrease in symptoms, decreased hospitalizations, improvement in fistula occurrence/healing) **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), secukinumab (Cosentyx), golimumab (Simponi), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), upadacitinib (Rinvoq), adalimumab

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(Humira), tocilizumab (Actemra), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), apremilast (Otezla)]

Ulcerative Colitis

- Member is ≥ 18 years of age **AND**
- Prescribed by or in consultation with a gastroenterologist **AND**
- Diagnosis of ulcerative colitis **AND**
- Member must have demonstrated a clinical response to therapy (i.e., decrease in symptoms compared to baseline such as stool frequency, rectal bleeding, and/or endoscopic activity, and/or tapering or discontinuation of corticosteroid therapy) **AND**
- Member is not on concurrent treatment with a TNF-inhibitor, biologic response modifier or other non-biologic agent [for example (list not all inclusive): abatacept (Orencia), secukinumab (Cosentyx), golimumab (Simponi), certolizumab (Cimzia), etanercept (Enbrel) infliximab (Remicade), rituximab (Rituxan), upadacitinib (Rinvoq), adalimumab (Humira), tocilizumab (Actemra), sarilumab (Kevzara), natalizumab (Tysabri), tofacitinib (Xeljanz or Xeljanz XR), apremilast (Otezla)]

Dosing recommendations:

Psoriasis

- Adults
 - For patients weighing 100 kg or less, the recommended dose is 45 mg initially and 4 weeks later, followed by 45 mg every 12 weeks.
 - For patients weighing more than 100 kg, the recommended dose is 90 mg initially and 4 weeks later, followed by 90 mg every 12 weeks.
- Pediatric patients 6-17 years old
 - Administer subcutaneously at Weeks 0 and 4, then every 12 weeks thereafter.

Body Weight of Patient at the Time of Dosing	Recommended Dose
less than 60 kg	0.75 mg/kg
60 kg to 100 kg	45 mg
more than 100 kg	90 mg

- For pediatric patients weighing less than 60 kg, the administration volume for the recommended dose (0.75 mg/kg) is shown in the table below; withdraw the appropriate volume from the single-dose vial.

Body Weight (kg) at the time of dosing	Dose (mg)	Volume of injection (mL)
15	11.3	0.12
16	12.0	0.13
17	12.8	0.14

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Body Weight (kg) at the time of dosing	Dose (mg)	Volume of injection (mL)
18	13.5	0.15
19	14.3	0.16
20	15.0	0.17
21	15.8	0.17
22	16.5	0.18
23	17.3	0.19
24	18.0	0.20
25	18.8	0.21
26	19.5	0.22
27	20.3	0.22
28	21.0	0.23
29	21.8	0.24
30	22.5	0.25
31	23.3	0.26
32	24	0.27
33	24.8	0.27
34	25.5	0.28
35	26.3	0.29
36	27	0.3
37	27.8	0.31
38	28.5	0.32
39	29.3	0.32
40	30	0.33
41	30.8	0.34
42	31.5	0.35
43	32.3	0.36
44	33	0.37

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Body Weight (kg) at the time of dosing	Dose (mg)	Volume of injection (mL)
45	33.8	0.37
46	34.5	0.38
47	35.3	0.39
48	36	0.4
49	36.8	0.41
50	37.5	0.42
51	38.3	0.42
52	39	0.43
53	39.8	0.44
54	40.5	0.45
55	41.3	0.46
56	42	0.46
57	42.8	0.47
58	43.5	0.48
59	44.3	0.49

Psoriatic arthritis

- Adults:
 - The recommended dose is 45 mg initially and 4 weeks later, followed by 45 mg every 12 weeks.
 - For patients with co-existent moderate-to-severe plaque psoriasis weighing more than 100 kg, the recommended dose is 90 mg initially and 4 weeks later, followed by 90 mg every 12 weeks.
- Pediatric patients 6 to 17 years old
 - Weight-based dosing is recommended at the initial dose, 4 weeks later, then every 12 weeks thereafter.

Pediatric Patients (6 to 17 years old) with Psoriatic Arthritis

Body Weight	Recommended Dose
Less than 60 kg	0.75 mg/kg
60 kg or more	45 mg
Greater than 100kg with co-existent moderate to severe plaque psoriasis	90mg

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Crohn's disease

- The recommended maintenance dosage is a subcutaneous 90 mg dose administered 8 weeks after the initial intravenous dose, then every 8 weeks thereafter.

Ulcerative colitis

- The recommended maintenance dosage is a subcutaneous 90 mg dose administered 8 weeks after the initial intravenous dose, then every 8 weeks thereafter.

Quantity Limits and duration:

- Initial therapy: Approve for 6 months
 - Psoriasis: quantity limit 1 injection/4 weeks for the first 2 fills, followed by 1 injection/12 weeks.
 - Psoriatic arthritis: quantity limit 1 injection/4 weeks for the first 2 fills, followed by 1 injection/12 weeks.
 - Crohn's disease: quantity limit 1 injection/8 weeks
 - Ulcerative colitis: quantity limit 1 injection/8 weeks
- Continuation of therapy: Approve for 1 year
 - Psoriasis: quantity limit 1 injection/12 weeks
 - Psoriatic arthritis: quantity limit 1 injection/12 weeks.
 - Crohn's disease: quantity limit 1 injection/8 weeks
 - Ulcerative colitis: quantity limit 1 injection/8 weeks

References:

1. Stelara Prescribing Information. Janssen Biotech, Inc. Horsham, PA. Updated November 2024.